

Breastfeeding: Assistance, Aids and Contraindications

Policy: Local agencies shall provide education, assistance and breastfeeding aids to directly support the initiation and continuation of breastfeeding.

Regulations: CFR 246.11 c, e, CFR 246.14 c (10), USDA FNS WIC Breastfeeding Policy and Guidance (July 2016), Colorado WIC Policy

Procedure/Additional Guidance:

Assistance with Breastfeeding Technique and Assessment

WIC staff can assist clients with breastfeeding techniques, positioning and latch within the following guidelines:

- Ask permission from the client to observe a breastfeeding session to assess positioning, latch and breastfeeding technique. (“Would you like me to offer some help on latching your baby?”)
- Verbally direct, use a doll to model, or a video to demonstrate/show various positions and latch techniques.
- If a client requests direct, hands-on assistance, staff shall use their hands to manipulate or place the baby. An effort should be made not to directly contact the client’s breast.

Health First Colorado (Medicaid) Reimbursement for Lactation Services

Federal policy prohibits individuals and organizations from receiving reimbursement and/or compensation from more than one federal funding source for the same time block (unit), including both individual and group session reimbursement.

- WIC staff may not be compensated/reimbursed from both Health First Colorado and the Colorado WIC grant for lactation services offered at the same time (block/unit).
- WIC staff may only bill for Health First Colorado reimbursement for lactation services provided outside of WIC paid FTE/time.
- WIC staff with advanced lactation credentials, who also provide lactation services reimbursed by Health First Colorado outside of WIC paid FTE/time, may do so if:
 - Health First Colorado reimbursement is not impeding upon WIC staff duties or services provided to WIC clients; and
 - Documentation for Health First Colorado reimbursements is kept separate from WIC’s Management Information System and stored on a separate charting platform, as data sharing between platforms is not allowed.

Test Weights

- WIC staff (High Risk Counselors, Educators, Breastfeeding Peer Counselors, WLS, DBE, CLC, CLE and IBCLC) should not perform test weights on infants to determine if the client’s milk is transferred. This procedure does not fit within routine or high risk WIC visits in that it requires a more thorough assessment (including feeding history, elimination patterns, breast variables, infant health status, duration of feeding problem, etc.) than time permits. WIC staff should refer infants who are not gaining weight well to their healthcare provider.
- WIC staff can weigh infants and offer clients to weigh their infants on a regular basis when the purpose is to assure the parent that their infant is growing.

Breastfeeding Aids

Breastfeeding aids describe items that directly aid in the removal of human milk from the breast and/or provide human milk to the infant. Breastfeeding aids that directly support the initiation and continuation of breastfeeding are an allowable WIC Program cost. See the *Breastfeeding: Breast Pump Issuance* policy and *WIC Lactation Specialist (WLS) Training Series* for more information.

- Breastfeeding aids and pumps can only be ordered on a quarterly basis through the State Office (see *Breastfeeding: Breast Pump Ordering and Maintenance* policy). Other allowable breastfeeding aids may be purchased with funds available in the local agency budget. (Note: Funds cannot be used to purchase breast pumps or pump supplies, as purchasing such supplies outside of state ordering/contracts is not permitted).
- Breastfeeding aids should be provided to WIC postpartum clients based on individual need and requested support, not as an inducement to consider or to continue breastfeeding.



- When breastfeeding aids are provided to clients, the State's *Breast Pump/Aid Release Form* must be reviewed with the client and a signature captured in the Compass computer system per policy outlined in *Breastfeeding: Breast Pump Issuance* policy.
- Liability - please see the *Breastfeeding: Breast Pump Issuance* policy.
- Only aids listed under the following table "Allowable Aids" are allowed for purchase with WIC funding by local agencies.

Allowable Aids	Non-Allowable Aids
• Milk storage bags	• Breast pumps/pump supplies outside of state ordering/contracts
• Nipple shells	• Nipple shields
• Supplemental Nursing Systems (SNS)	• Topical creams, ointments, vitamin E, other medicinal
• Nursing bras and pads	• Foot stools
	• Nursing pillows or clothing
	• Baby bottles

Nipple Shields

WIC staff cannot issue and should not place nipple shields on clients (not to be confused with breast shields or flanges used with a breast pump). The WIC clinic and scheduling does not provide adequate timing and resources to perform the necessary support and follow up. If use of a nipple shield is warranted, clients should be referred to a lactation consultant in the community.

Nipple Shells

WIC staff trained in lactation management (WLS, DBE, CLC, CLE, IBCLC) can provide nipple shells to clients as needed. Nipple shells can be used for:

- Sore nipples: worn over the nipples between feeds to minimize contact with clothing and allow healing.
- Flat or inverted nipples: worn to press around the base of the nipple to cause the nipple to protrude.
 - ✓ Prenatal use: worn in the last month of pregnancy. Clients wearing shells prenatally **MUST** get permission from their obstetric care provider as shells can trigger contractions of the uterus.
 - ✓ Postpartum use: worn approximately 30 minutes before each feeding.

Supplemental Nursing Systems (SNS):

WIC staff trained in lactation management (WLS, DBE, CLC, CLE, IBCLC), ideally those with advanced skills only (IBCLC), may provide supplemental nursing systems to clients as needed. Typically, SNS are only issued after first prescribed by the hospital/healthcare provider. SNS can be used for:

- Underweight breastfed infants.
- Low milk supply.
- Re-lactating clients.
- Clients attempting to lactate for an adopted infant.
- Infants with ineffective suck or latch due to medical concerns/issues.

Contraindications

There are very few medical reasons when a client should not breastfeed. Identify contraindications that may exist for the client. [See WIC Lactation Specialist \(WLS\) Training Series](#) for additional details.

Breastfeeding is contraindicated when:

- The infant is diagnosed with classic galactosemia, a rare genetic metabolic disorder.
- The client has tested positive for HIV (human immunodeficiency virus) or has acquired immune deficiency syndrome (AIDS).
- The client has tested positive for human T-cell lymphotropic virus type I or type II (HTLV-1/2).
- The client is using illicit street drugs, such as PCP (phencyclidine) or cocaine (exception: narcotic-dependent individuals who are enrolled in a supervised methadone program and have a negative screening for HIV and other illicit drugs can breastfeed).



Breastfeeding may be **temporarily contraindicated** when (clients should be provided breastfeeding support and a breast pump, if eligible, and may resume breastfeeding after consulting with a physician to determine when their breast milk is safe for their infant and should be provided with lactation support to establish and maintain a milk supply):

- The client is infected with untreated brucellosis.
- The client has an active herpes simplex virus (HSV) infection with lesions present on the breast.
- The client is undergoing diagnostic imaging with radiopharmaceuticals.
- The client is taking certain medications or illicit drugs.

Direct breastfeeding may be **temporarily contraindicated** and the client should be temporarily isolated from their infant(s) (should be provided breastfeeding support and a breast pump, if eligible) but expressed breast milk can be fed to the infant(s) when:

- The client has untreated, active tuberculosis (TB) (may resume breastfeeding once they have been treated for approximately two weeks and is documented to be no longer contagious).
- The client has active varicella (chicken pox) infection that developed within the five days prior to delivery to the two days following delivery.